# Two Hospitals on One Street

Narrated accessibility walk: Harvard station to Spaulding Rehabilitation Hospital Cambridge and CHA Cambridge Hospital, Cambridge St, Cambridge, MA

Pilot reports 4 and 5, combined | Recorded October 27, 2026

Part of an independent series of narrated walks that check whether people with disabilities can reach clinical research sites. Prepared by Uri Kartoun, PhD, Founder, DBbun LLC, Cambridge, MA | uri.kartoun@dbbun.com | dbbun.com

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## Summary

On one morning, a narrated walk followed the trip a research participant might make from the Red Line at Harvard station along Cambridge St to two hospitals about 500 m apart: Spaulding Rehabilitation Hospital Cambridge and CHA Cambridge Hospital. The walker then watched how people arrive at each entrance (about 9 minutes at Spaulding and half an hour at CHA).

- **Same street, very different arrivals.** Spaulding has two bays reserved for ambulances and wheelchair vans, a quiet drive, automatic doors and a bench at the door. At CHA, the drop-off loop is too small: vans wait, cars stop in the travel lane, a short exit signal backs traffic up, and people with mobility aids take unsafe shortcuts. The observations suggest that arrival design and traffic management, rather than the street itself, deserve a closer look. The comparison is not like for like: CHA has an emergency department, far more arrivals and a different patient mix, and it was watched for about half an hour against about 9 minutes at Spaulding.
- **Three of the five high-severity items are at CHA:** no curb ramp on the line people walk from the entrance toward the garage, where a man with a rollator could not cross (C15); shortcuts across the traffic island (C16); and an overloaded drop-off loop (C17). The other two are on the Spaulding side: the broken warning strip at Harvard Square (S5) and tree roots on the path to the outpatient center (S22).
- **On the way from Harvard station, the approach is the burden:** a broken tactile warning strip at Harvard Square, brick sections and a pothole on Cambridge St, and about 4 minutes of walking with no bench.

- **A test of the open-data screen:** both hospitals were screened with public data before the walk, and the screen was re-run afterwards on the exact stretch walked. It predicted the brick share and the gentle slope, missed a seatless gap, flagged curb ramps the walker found in good shape, and missed every day-of defect. It also named its own blind spot in advance, hospital parking and drop-off, which is where CHA's problems are.
- **50 items in all:** 32 possible barriers (5 rated high), 17 positive features and 1 note. Outdoors only: by protocol, the walker does not enter care facilities without an appointment or an invitation.

**Disclaimer.** This report was produced with the help of AI tools (automatic speech transcription and AI-assisted image analysis; see "How this report was produced") and was reviewed by the author. Errors and omissions are possible. Items are possible barriers observed on one day, not legal compliance determinations or findings of violation. Times and distances are estimates. Observations reflect conditions on October 27, 2026 and may have changed since. People and vehicles are not identified: faces, license plates and company logos are blurred. This is an independent project; it is not affiliated with or endorsed by either hospital.

## Walk details

| | |
|---|---|
| **Date** | October 27, 2026 (Tuesday), morning, sunny and dry |
| **Route** | Harvard station elevator, Harvard Square, Harvard Yard, Quincy St, then Cambridge St east past Felton St, Irving St, Trowbridge St and Roberts Rd to Spaulding (about 1.2 km); then Cambridge St to CHA and around its campus |
| **Recordings** | Video: Harvard station to Spaulding (19 min 33 s); Spaulding to CHA and around the campus (13 min); six short videos at the CHA entrance (about 15 min). Audio only: the Spaulding entrance and grounds (8 min 36 s) and three narrations at the CHA entrance (15 min 23 s). 11 photos at CHA. |
| **Narrator** | Uri Kartoun, PhD (DBbun LLC); assessed the route from the point of view of wheelchair, walker and cane users, blind and low-vision people, and people who tire easily. The walker does not use a mobility aid. |
| **Why these sites** | Spaulding runs clinical research with people with disabilities; CHA Cambridge Hospital is a public safety-net hospital on the same street, serving many patients who rely on transport services. Harvard station is the nearest Red Line station to both. |
| **Scope** | Outdoors only. Item numbers: S = Harvard station to Spaulding and the Spaulding grounds; C = CHA Cambridge Hospital. |

## Two hospitals side by side

What a participant with a disability meets on arrival at each hospital, from the items in the Findings section. Read with care: the two sites differ in volume and patient mix, and CHA was watched for longer.

| | **Spaulding Rehabilitation Hospital Cambridge** | **CHA Cambridge Hospital** |
|---|---|---|
| **Setting** | Rehabilitation hospital; quiet drive, few vehicles | Public hospital with an emergency department; busy drive |
| **Drop-off** | Two bays reserved for ambulances and wheelchair vans, off the street (S21) | Loop too small: vans wait, cars stop in the travel lane (C17, C18) |

| | Spaulding Rehabilitation Hospital Cambridge | CHA Cambridge Hospital |
|---|---|---|
| **Leaving by car or van** | No queues seen | Short green onto Cambridge St; cars back up into the loop (C19) |
| **Entrance doors** | Automatic (S17); sensor doors at neighboring buildings (S19) | Automatic at the Emergency entrance (C13); a side door without an opener (C23) |
| **Seating at the door** | Bench at the entrance, garden with seating (S17, S19) | Benches at the entrance (C13) |
| **Crossings near the entrance** | No problems seen at the main entrance; tree roots on the path to the outpatient center (S22) | No curb ramp on the direct line to the garage (C15); people cut across the traffic island (C16) |
| **Accessible parking** | 4 spaces at the main entrance, 4 at the outpatient center; door room on one side only for 3 (S20, S23) | Accessible spaces on campus (C4); garage ramp too steep for wheelchairs (C20); patients pay at most $10 (C22) |
| **Air at the entrance** | Ambulances and vans idling (S18) | Smoking next to the tobacco-free signs (C12) |
| **Signs** | Clear (S23) | Clear (C2) |
| **Sidewalk outside** | Construction at the frontage, sidewalk open (S16) | Heavily patched sidewalk, a sign on a broken concrete block, a truck parked on the sidewalk (C7, C9, C10) |
| **Overall (one visit)** | Few obstacles seen at the entrance; interior not assessed | Arriving by car or van was the main difficulty; rollator and cane users met a missing curb ramp and took shortcuts |

## What travelers would meet

What each group of travelers would likely meet, based on what the walker observed. These are not test results: no traveler with these disabilities made the trip, so each cell notes what was seen and, where it matters, what remains uncertain.

| Traveler | Harvard station to Spaulding | At Spaulding | At CHA |
|---|---|---|---|
| **Wheelchair (manual or power)** | Seen: a working elevator at Harvard; about 1.2 km with brick sections, a broken warning strip and a sidewalk pothole (S5, S11, S12). Uncertain: the effort in a manual wheelchair; curb ramp slopes | Seen: automatic doors, van bays, accessible parking; tree roots on the outpatient path (S22). Uncertain: the interior | Seen: a crowded drop-off loop; no curb ramp on the direct line to the garage (C15, C17). Uncertain: whether the step-free way around is signed |
| **Walker, rollator or cane** | Seen: broken bricks, a pothole, wet leaves (S8, S12, S13) | Seen: a bench at the entrance; tree roots on the outpatient path (S22) | Seen: a man with a rollator could not cross (C15); people with canes cut across the island (C16) |
| **Blind or low vision** | Seen: audible signals; a broken warning strip at Harvard Square (S5). Uncertain: wayfinding through Harvard Yard | Seen: clear visual signs. Uncertain: tactile wayfinding; the interior | Seen: an audible garage-exit warning; cars stopping in the lane and cutting around the island; a sign block with sharp edges (C9, C18) |
| **People who tire easily or are sensitive to smoke or fumes** | Seen: about 19 minutes, with no bench for about 4 minutes (S14). Uncertain: bus 69 as an alternative (not tested) | Seen: benches and a garden; vehicles idling by the entrance bench (S18) | Seen: benches at the entrance; smoking by the tobacco-free signs (C12); no bench on the way from Spaulding (C1) |

## Testing the open-data screen

Before the walk, on October 23, 2026, both hospitals were screened automatically using only public data (OpenStreetMap, USGS elevation, City of Cambridge open data on curb ramps, sidewalks, crashes and service requests, and the MBTA), from Harvard station and from the nearest bus 69 stops. The screen's own route from Harvard station went through Broadway and Prescott St, while the walker went through Harvard Yard. To compare like with like, the same screen was re-run after the walk, unchanged, on the Cambridge St stretch actually walked (Quincy St to Spaulding, 609 m). The Harvard Yard paths are not in the city's sidewalk data, so that part is not compared. On the screen's original route, the figures were 46% brick and 667 m without a mapped seat.

| Screen prediction (Spaulding unless marked CHA) | What the walk found | Result |
|---|---|---|
| **About 40% of the walked Cambridge St stretch on brick** | About 35 to 40% of the same stretch is brick (frame count) | Confirmed |
| **Longest stretch without a mapped seat: 229 m** | No bench from Quincy St to just past Trowbridge St, about 360 m by the screen's own distances | Not confirmed: one mapped seat was not seen (it may be private, gone, or not a bench) |
| **14 of 34 curb ramps rated a problem by the city (surveys from 2013 and 2023): slope at Broadway and Prescott St, Sumner St and Trowbridge St; puddling at Irving St; fair condition at Felton St and Roberts Rd** | The walker found these crossings in good shape. It was a dry day, so puddling could not be seen; slope was not measured | Not confirmed on a dry day; slope needs measuring |
| **Gentle elevation; slope not the main barrier** | No hills noted | Confirmed |
| **No on-street accessible parking near Spaulding; hospital parking and drop-off are not in any open dataset, so check on site** | Accessible spaces at two Spaulding entrances, plus two van and ambulance bays | Resolved on site |
| **For people who tire easily, bus 69 is the realistic option: stops 23 to 74 m from the entrance, on concrete** | Not tested on this walk | Not tested |
| **CHA: Harvard station to CHA is 1,461 m with 25 curb ramps rated a problem; hospital parking and drop-off are not in any open dataset, so check on site** | The most serious CHA problems are all in the arrival area: drop-off, cars stopping in the lane, island and a missing curb ramp (C15 to C18) | The screen named its own blind spot, and that is where the problems were |
| **CHA: 18 past traffic or pedestrian-signal complaints within 25 m of the short routes from the bus 69 stops to the entrance (311 data, all years)** | A short exit green onto Cambridge St backs cars up into the drop-off loop (C19) | Possibly related (whether the complaints concern the same signal is not known) |
| **CHA: bus 69 stops 123 to 157 m from the entrance, on concrete, with no problem curb ramps** | Not tested on this walk | Not tested |
| **(Not predictable from data)** | Broken warning strip (S5), broken bricks (S8), sidewalk pothole (S12), no elevator arrival light (S2), misleading elevator-status screen (S3), construction (S16), tree roots (S22), idling vehicles (S18) | Missed by the screen |

What this shows: the screen got the lasting features of the route right (surface type and slope) and pointed to where to look. Its seat data missed at least one gap, its curb ramp ratings did not match what the walker saw on a dry day, and it cannot see day-of conditions. Most usefully, it named its own blind spot in advance (hospital parking and drop-off), and that is where the most serious problems turned out to be. The method therefore uses both steps: the screen to plan, the walk to confirm and to cover what the data cannot see.

# Findings

Severity notes appear in the barrier column; “positive” marks features that worked well. Conf. (confidence): H = seen and recorded on video or photo, or confirmed by data; M = described in the narration only (parts were recorded as audio) or not measured.

## A. Harvard station to Spaulding

| # | Where | Possible barrier or feature | Observation | Most affected | Conf. | Suggested fix |
|---|---|---|---|---|---|---|
| S1 | Harvard station platform | Elevator sign visible (positive) | A sign to the elevator is visible from the platform. One exit toward the buses is stairs only, but the elevator route is signed. | - | H | - |
| S2 | Harvard station elevator to the street | No arrival light | Glass elevator, clean, no bad smell (positive). The call button gives a steady light, but there is no light or sound when the car arrives; you can see it coming only because the shaft is glass. | Deaf or hard of hearing; low vision | H | Add an arrival light and chime |
| S3 | Harvard station, digital elevator-status screen | Misleading status for Davis | The screen (with a press-for-audio button, positive) lists elevator outages elsewhere. Davis is shown as working, but Davis has no elevator at its Holland St entrance, so a traveler planning a trip could be misled. | Wheelchair users planning a trip to Davis | H | Show entrances without elevators, not only outages |
| S4 | Harvard Square and Cambridge St at Quincy St | Accessible pedestrian signals (positive) | Push buttons with audible signals at both crossings; the walk phase was announced. | - | H | - |
| S5 | Harvard Square crossing | Broken tactile warning strip (High) | The red tactile warning strip at the curb ramp is badly broken, with holes and missing pieces, and the surface around it is cracked. | Blind and low vision (warning lost); wheelchair; cane | H | Replace the warning strip and repair the ramp surface |
| S6 | Harvard Square, construction area | Pothole at a crossing edge; barriers | A pothole at the edge of a crossing; construction barriers along a brick sidewalk. Passable at the time. | Wheelchair, walker | H | Patch the pothole; keep a clear signed path past the work |
| S7 | Harvard Yard | Flat paths with benches (positive) | Mostly flat brick and paved paths with benches; repaired root damage. | - | H | - |

| # | Where | Possible barrier or feature | Observation | Most affected | Conf. | Suggested fix |
|---|---|---|---|---|---|---|
| S8 | Harvard Yard path | Broken bricks around a utility cover | Broken and missing bricks around a utility cover in the path. | Wheelchair casters, cane, walker | H | Reset the bricks flush with the cover |
| S9 | Harvard Yard and Quincy St | Shared paths with cyclists | Many cyclists on shared paths; the walker had to stay alert. | Low vision; slow walkers | M | Signs asking cyclists to slow down on shared paths |
| S10 | Church and design-school buildings on the route | Push-button doors (positive) | Accessible entrances with push-button openers. | - | H | - |
| S11 | Cambridge St sidewalks | Brick sections | About 35 to 40% of the Cambridge St stretch is brick (by frame count), some uneven from tree roots (“the bricks are more difficult”). | Wheelchair, walker; people with pain or fatigue | H | Reset uneven bricks; smooth concrete strip in the main walking line |
| S12 | Cambridge St near Felton St | Sidewalk pothole; tree-lifted panel | A pothole in the sidewalk, and a concrete panel lifted by a tree that has been partly ground down. | Wheelchair, walker, cane | H | Patch the pothole; finish grinding or replace the panel |
| S13 | Cambridge St | Wet leaves | Thick wet leaves on the sidewalk (seasonal). | Anyone with balance or mobility problems | H | Leaf clearing on main walking routes to hospitals |
| S14 | Cambridge St, Quincy St to Trowbridge St | No bench for about 4 minutes of walking | No bench from Quincy St until just past Trowbridge St (“the first time that we see a bench”), about 4 minutes at the walker’s pace and longer for a slow walker. More benches after that, outside shops. | People who need to rest | H | Add at least one bench on this stretch |
| S15 | Cambridge St crossings, Felton St to Roberts Rd | Curb ramps in good shape (positive) | The walker described these crossings as “good shape” and “flat, good quality”; a store selling mobility aids is on the route. | - | M | - |
| S16 | Spaulding frontage, Cambridge St | Construction at the frontage | Spaulding sign visible from the street (positive). Construction fencing, cones and tape along the frontage; the sidewalk was still open and wide. | Low vision (cones); wheelchair if the work expands | H | Keep the sidewalk clear and the work edge marked |

## B. Spaulding entrance and grounds

| # | Where | Possible barrier or feature | Observation | Most affected | Conf. | Suggested fix |
|---|---|---|---|---|---|---|
| S17 | Main entrance | Automatic doors and a bench (positive) | Automatic doors; a bench right by the entrance, in use. The walker: “highly accessible”. | - | M | - |

| # | Where | Possible barrier or feature | Observation | Most affected | Conf. | Suggested fix |
|---|---|---|---|---|---|---|
| S18 | Entrance drive | Vehicles idling at the door | Ambulances and transport vans (one with a wheelchair symbol) waited at the entrance with engines running over several minutes, next to the entrance bench. | People with asthma or chemical sensitivity; anyone on the entrance bench | M | No-idling signs; ask transport providers to switch off engines |
| S19 | Beside the entrance and neighboring buildings | Garden, sensor doors, café (positive) | A walkway and garden with a small fountain; flat approaches with sensor-operated doors and benches at the neighboring buildings on the site; a café. | - | M | - |
| S20 | Parking by the main entrance | Accessible spaces: door room on one side only | Four signed accessible spaces. Only one has room to open doors fully on both sides; the other three have room on one side only. | Wheelchair users, as drivers or passengers, and ramp vans | M | Mark which side each access aisle serves; add a both-sides space |
| S21 | Parking by the main entrance | Bays for ambulances and wheelchair vans (positive) | Two spaces reserved for ambulances and wheelchair vans: a defined loading point off the street. | - | M | - |
| S22 | Site path toward the outpatient center | Tree roots across the path (High) | Tree roots make this stretch “extremely difficult to cross with a wheelchair”; the walker used the site road instead. | Wheelchair, walker, cane | M | Repair the path over the roots, or sign a step-free route |
| S23 | Site road and outpatient center | Clear signs; accessible spaces (positive) | Signs to the Main Entrance, Ambulance Entrance and Outpatient Center; “Do not enter, use main entrance in front” at a side door. Four more accessible spaces at the outpatient center, whose entrance was “highly accessible”. | - | M | - |
| S24 | Rear service area | Unmarked steep ramp | A steep ramp to a door, apparently for equipment, with no sign saying it is not a public entrance. | Wheelchair users who take it for an entrance | M | Sign it “Service only; accessible entrance at …” |
| S25 | Side path at the frontage | Cracks; temporary cones | Cracks in a side path; cones around a renovation area. | Wheelchair, walker | H | Repair cracks when the renovation ends |

## C. CHA Cambridge Hospital: approach and campus perimeter

| # | Where | Possible barrier or feature | Observation | Most affected | Conf. | Suggested fix |
|---|---|---|---|---|---|---|
| C1 | Cambridge St, Spaulding to CHA (Lowell St and Leonard Ave area) | No benches on this stretch | Accessible crossing (positive), but no benches between the two hospitals. | People who need to rest | M | Add a bench |

| # | Where | Possible barrier or feature | Observation | Most affected | Conf. | Suggested fix |
|---|---|---|---|---|---|---|
| C2 | CHA campus, Emergency side and entrances | Clear signs (positive) | Clear “Emergency”, “Main Entrance”, “Not an entrance” and “Ambulance only” signs, with arrows; a signalized crossing nearby. | - | H | - |
| C3 | Path to a CHA building | Slightly steep path | Accessible but “a little bit steep”. | Wheelchair, walker | M | Check the slope; add a handrail if steep |
| C4 | CHA campus | Accessible parking; transport vans (positive) | Signed accessible parking spaces; several wheelchair-accessible transport vans in use on the campus. | - | H | - |
| C5 | Near the Macht building | Torn surface | A torn and damaged walking surface (“they will need to fix that”). | Wheelchair, walker, cane | M | Repair the surface |
| C6 | Macht building | Ramp, seating and signs (positive) | Wheelchair ramp; outdoor tables and seating; signs to Emergency and the Main Entrance. | - | H | - |
| C7 | Street beside the hospital, by a construction area | Delivery truck parked on the sidewalk | A delivery truck parked on the sidewalk next to a construction area, narrowing the path. | Wheelchair, walker, stroller, low vision | H | Loading zone off the sidewalk; enforcement |
| C8 | Side ramp near construction | Ramp looks unsafe | Because of the construction, the ramp looked unsafe; the walker did not use it. | Wheelchair | M | Close it with a signed alternative, or make it safe |
| C9 | Cambridge St, hospital frontage | Sign on a broken concrete block | A “CHA is tobacco free” sign mounted on a broken concrete block with very sharp edges, beside the sidewalk (“neglected… dangerous”). | Blind and low vision (a cane may miss it), children | H | Repair or remove the block |
| C10 | Cambridge St sidewalk along the hospital | Heavily patched sidewalk | Many patches (concrete or rubbery fill); passable for a wheelchair, but the walker would ask for the whole stretch to be replaced. | Wheelchair, walker | H | Replace the sidewalk (City of Cambridge or hospital, depending on ownership) |
| C11 | Crossing near the entrance | Crossing closed (temporary) | A crossing appeared closed with a caution sign; a pedestrian crossed beside it anyway. | Wheelchair, low vision | M | Signed alternative crossing while closed |
| C12 | Near entrances, by “CHA is tobacco free” signs | Smoking by the signs | Many tobacco-free signs (positive), but two people were seen smoking next to them, one about 30 ft from an entrance, while a person with a walker, helped by staff, passed slowly through the smoke. A smoking area exists near Cambridge St but was not used. | People with asthma or chemical sensitivity; slow walkers who cannot move away | M | Staff reminders; move the smoking area further from paths and entrances |

## D. CHA Cambridge Hospital: main entrance, drop-off and garage

| # | Where | Possible barrier or feature | Observation | Most affected | Conf. | Suggested fix |
|---|---|---|---|---|---|---|
| C13 | Emergency entrance | Automatic doors and benches (positive) | Automatic doors and benches; a person with a cane walked out easily. | - | M | - |
| C14 | Lobby (seen through the window) | Registration and Financial Assistance (positive) | Registration and Financial Assistance desks visible from outside; about 20 people waiting, including people with rollators and crutches. Sun glare on the glass made signs hard to read from outside. | - | M | - |
| C15 | Crossing from the entrance toward the garage and the other hospital building | No curb ramp on the direct line (High) | A man who walks with great difficulty, using a rollator and helped by a companion, tried to cross here and could not: the curb on the far side is raised, with no ramp. The marked crosswalk lands further along, so people who cannot step up must go around. | Rollator, wheelchair, walker | H | Add a curb ramp on this line, or a sign at the entrance pointing to the step-free route |
| C16 | Traffic island in the entrance drive | Shortcuts across the island (High) | A woman with a cane, and later another person, cut across the island instead of using the marked crossings, stepping into the path of cars. The walker: the island is “kind of steep and dangerous”. | Cane, walker, low vision | M | Find where people cross and add a direct step-free crossing there; avoid barriers that would lengthen the walk |
| C17 | Entrance drop-off loop | Drop-off area too small (High) | During about half an hour at the entrance: up to 4 transport vans waiting at once; private cars and ride-share vehicles stopping in front of the entrance or in the travel lane; about 4 cars queuing in from Cambridge St; at most 3 cars can pass. Some drivers dropped off inside the garage instead. “Very difficult to drop patients properly.” | Wheelchair and walker users being dropped off; transport vans | H | Marked drop-off bays; a drop-off attendant at peak times; directions to drop-off in the garage |
| C18 | Travel lane by the island | Vehicles waiting in the travel lane by the island | Cars waited for several minutes in the travel lane beside the island, which is not a stopping area (the opposite curb is painted “Fire lane” and “No parking”), blocking cars and a taxi leaving the garage; drivers honked and one had to reverse. | Everyone leaving by car or van; pedestrians crossing between stopped cars | H | “No stopping” signs; enforcement |

| # | Where | Possible barrier or feature | Observation | Most affected | Conf. | Suggested fix |
|---|---|---|---|---|---|---|
| C19 | Exit signal onto Cambridge St | Short green for vehicles leaving | Timed from the video: about 10 seconds of green and a few seconds of amber, then about 75 seconds of red, with no turn on red (one full cycle; frames 3 seconds apart). Cars leaving the loop backed up into the drop-off area while they waited. | Vans and cars carrying patients | H | Review the signal timing with the city |
| C20 | Garage entrance | Steep ramp shared by cars and people | A steep ramp into the garage shared by cars and pedestrians (“definitely not for wheelchairs”); sign: “Caution pedestrian crosswalk top of ramp”. | Wheelchair, walker | M | Signed step-free pedestrian route to the garage elevators |
| C21 | Garage exit | Audible warning (positive) | “Caution vehicles exiting” sign with an audible warning. | - | H | - |
| C22 | Garage | Patient parking capped (positive) | Rates posted from free (first 10 minutes) to $40 for 6 to 24 hours; patients pay at most $10. | - | M | - |
| C23 | Side door | No push-button opener | A side door without a push-button opener; signs send people to the main entrance. | Wheelchair, walker | M | Opener, or a clear sign to the nearest accessible door |
| C24 | Entrance curb | Good drop-off (positive) | A ride-share driver stopped by the door with hazard lights, let a person with a walker out, then parked in the garage. A transport van loaded a wheelchair user by ramp at the curb when the space was free. | - | M | - |
| C25 | Entrance drive and crosswalk | Utility markings; temporary barriers | Fresh yellow utility markings on the crosswalk and drive, which may mean work is planned, and orange plastic barriers in the drive (purpose not clear). | - | H | If work is planned, add the missing curb ramp (item C15) at the same time |

# Evidence for the priority findings

Where each priority finding can be checked in the recordings. The recordings are kept by the author and are not published, because they show faces and license plates.

| # | Finding | Where to find it (file, time) |
|---|---|---|
| S5 | Broken tactile warning strip, Harvard Square | Video IMG_6454, 3:12–3:15 (photo figure) |
| S12 | Sidewalk pothole near Felton St | Video IMG_6454, 14:09–14:26 (photo figure) |
| S14 | No bench, Quincy St to Trowbridge St | Video IMG_6454, 11:54–16:09 |
| S18 | Vehicles idling at the entrance | Audio spaulding_audio_2.m4a, 0:44–4:00 (narration only) |

| # | Finding | Where to find it (file, time) |
|---|---|---|
| S22 | Tree roots on the path to the outpatient center | Audio spaulding_audio_2.m4a, 5:33–5:46 (narration only) |
| C9 | Sign on a broken concrete block | Video IMG_6455, 9:47–10:17 |
| C12 | Smoking next to the tobacco-free signs | Audio camelia_ave.m4a, 0:00–1:50; audio hovey_ave.m4a, 0:00–0:13 (narration only) |
| C15 | No curb ramp; a man with a rollator could not cross | Seen and narrated as it happened: audio camelia_ave.m4a, 2:35–3:34. Explained on site afterwards: video IMG_6457, 0:08–1:13. The curb: photo IMG_6465. The man himself was not filmed. |
| C16 | Shortcuts across the traffic island | First person: audio hovey_ave.m4a, 3:41–4:53, and video IMG_6456, 0:15 –1:14. Second person: video IMG_6472, 5:24–5:47. The island: photo IMG_6469. |
| C17 | Drop-off loop too small | Audio cha_cambridge_hospital.m4a, 0:00–3:38; videos IMG_6458 to IMG_6460; video IMG_6472, 0:00–4:59; photo IMG_6467 |
| C18 | Vehicles waiting in the travel lane by the island | Videos IMG_6458 (0:00–0:39), IMG_6459, IMG_6460; video IMG_6472, 0:00–3:31 |
| C19 | Short exit green onto Cambridge St | Video IMG_6472: signal visible from 7:24 to 8:57 (frames 3 seconds apart); narration 6:23–9:17 |

## Questions for the hospitals and the city

**Spaulding.** Is the front desk usable from a seated position? Where is the nearest accessible restroom to the entrance? Are loaner wheelchairs available at the door? Is there a quiet waiting area? Are there signs from the lobby to research and clinic areas? Could the path to the outpatient center be repaired over the roots (S22), and could transport providers be asked to switch off engines while waiting (S18)?

**CHA Cambridge Hospital.** Could drop-off bays, “no stopping” signs along the island and an attendant at busy times be added (C17, C18)? Could a curb ramp be added on the line from the entrance toward the garage (C15), perhaps with the work the fresh utility markings suggest? Could a direct, step-free crossing be added where people already cut across the traffic island (C16), rather than barriers that would lengthen their walk? Could the smoking area be moved away from entrances and paths (C12)?

**City of Cambridge.** Could the warning strip at Harvard Square (S5) and the Cambridge St sidewalk defects (S12, C10) be repaired, a bench be added between Quincy St and Trowbridge St (S14), and the exit signal timing at CHA be reviewed (C19)?

**Study teams at both hospitals.** Do participants with disabilities get directions to the right entrance, drop-off point and parking before a visit?

## Checking the AI-assisted analysis

The item lists were drafted with AI help from the transcribed narration, the face-blurred frames and the photos, and reviewed by the walker. Corrections from that review: a video first assumed to show Spaulding mostly covers CHA; the audio files’ names were the phone’s automatic location labels, so their content decided where each belongs; the transcription heard “CJ” for CHA and “stroller” for a rollator, and an unfamiliar word was confirmed by the walker to mean crutches. The key CHA claims in the narration were checked against the videos and photos (Figures 5 to 8), and the exit signal (C19) was timed from the video. Automatic face blurring had

blurred some sign text in Figure 4, so that frame was taken again from the original video. License plates and van-company logos were blurred by hand, and the narration's mention of a plate number was left out.

## Photo evidence

Video frames (Figures 1 to 4 and 8) and photos (Figures 5 to 7); faces, license plates and company logos are blurred.



**Figure 1.** Harvard Square: the tactile warning strip at a curb ramp, badly broken (S5).



**Figure 2.** Cambridge St near Felton St: a pothole in the sidewalk (S12).



**Figure 3.** Cambridge St just past Trowbridge St: the first bench after about 4 minutes of walking (S14).




**Figure 4.** The Spaulding sign at 1575 Cambridge St, with construction cones along the frontage (S16).

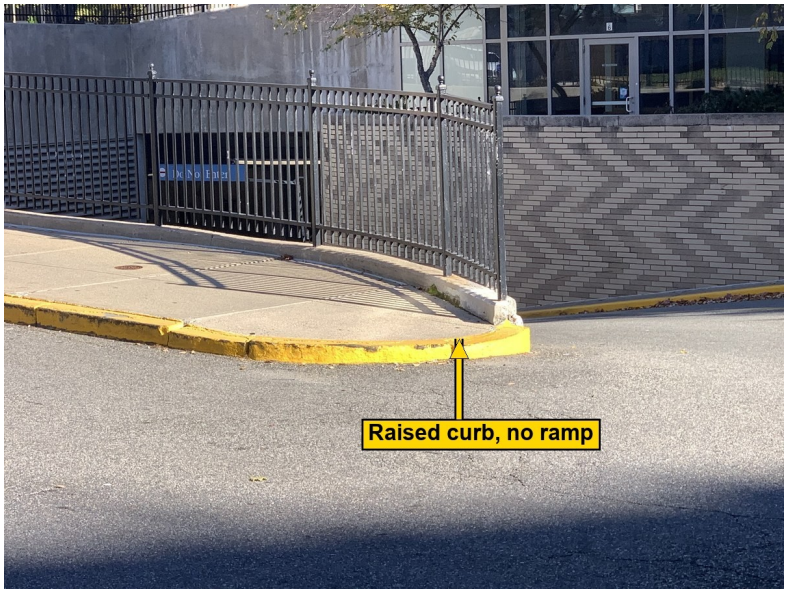



**Figure 5.** CHA: the raised curb with no ramp on the line from the entrance toward the garage, where a man with a rollator could not cross (C15).




**Figure 6.** CHA: the traffic island in the entrance drive, where people with canes cut across (C16).




**Figure 7.** CHA: the drop-off loop, with transport vans and cars stopped along the island (C17).




**Figure 8.** CHA: a car (the dark SUV) standing for several minutes in the travel lane beside the island, which is not a stopping area; the cars behind it and traffic leaving the garage could not get past (C18).

## How this report was produced

- **Capture:** narrated smartphone videos, audio recordings and photos made by the author.
- **Local processing on the author's computer:** audio extracted with ffmpeg (imageio-ffmpeg 7.1); narration transcribed with faster-whisper, model medium.en, on a local GPU; one frame sampled every 3 seconds; faces blurred with OpenCV 4.14 before any image left the computer; license plates and logos blurred by hand in the figures.
- **AI review:** the blurred frames, the photos and the transcripts were reviewed by Claude (Anthropic), model claude-opus-5-5, which drafted the item lists and this report. The author reviewed the findings.
- **Open-data screen:** OpenStreetMap, USGS elevation and City of Cambridge open data, processed with Python (OSMnx, GeoPandas) on the author's computer.
- **Limitations:** one walker on one dry morning, who does not use a mobility aid; times, distances and vehicle counts are estimates and slopes were not measured; parts were recorded as audio only (Conf. M); building interiors were not assessed; items are possible barriers, not legal compliance determinations.